Ysbyty Gwynedd
Anaesthetics Department
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CRITICAL CARE

Tracheostomy Care

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Key Points

- Care of patients with tracheostomy must be discussed with a consultant anaesthetist
- All patients with cuffed tracheostomies must be discharged to critical care
- Care of patients with uncuffed tracheostomies must occur on approved wards; Dulas, Moelwyn, Prysor.
- NTSP Bead Head signs must be displayed, and appropriate emergency equipment (blue box and spares) must be at the patient's bedside at all times.
- Tracheostomy emergencies should be managed as per National Tracheostomy Safety Project (NTSP) guidance.

Introduction

Tracheostomies are a key but less common method of managing an airway. Safe management requires staff to understand the anatomy, care processes and emergency protocols to allow safe care within anaesthesia and critical care.

Tracheostomy Care - Staffing

All cases involving patients with an existing tracheostomy must be discussed with a consultant. All cases involving the insertion of a new cuffed tracheostomy should be lead by a consultant anaesthetist, but on-site supervision is acceptable for staff with appropriate skills.

All staff leading the care of patients with tracheostomies should undertake documented training, specific to their role. For anaesthetists this should be supervised practise usually part of an airway block and NTSP modules. Critical care nurses should complete the All Wales Tracheostomy Competency assessment document or NTSP modules[1].

Tracheostomy Care – Equipment & Organisational Management

All areas caring for tracheostomy patients must have appropriate spares and emergency equipment as set out by the NTSP[2]. NTSP Bed Head Signs must be displayed. The presence of these must be confirmed during handover of the patient.

Surgical tracheostomies for critical care patients should be arranged using the Surgical Tracheostomy LocSSIP, available on the departmental website.

Patients with cuffed tracheostomy tubes must be transferred from theatres to critical care post-operatively for management in an area with suitably trained staff.

Recovery from anaesthesia for uncuffed tracheostomies should occur in theatre, and any stay in recovery should be supported by the anaesthetist.

Uncuffed Tracheostomy patients must be discharged to wards approved for the care of tracheostomies. The settings may be varied by site management, but usually are limited to Critical Care, Dulas Ward (ENT), Moelwyn Ward (respiratory medicine), Prysor Ward (Stroke Ward).

Tracheostomy Care – Emergency Care

Tracheostomy emergencies carry a high mortality, even when managed according to national guidelines. Local simulation traing is regularly run as part of the ICU airway training sessions. For information please contact Dr. Will Sutcliffe.

All tracheostomy emergencies should be managed according to the National Tracheostomy Safety Project guidance[3].

References

1) https://tracheostomy.org.uk/e-learning
2) https://tracheostomy.org.uk/storage/files/Adult%20bedside%20equipment.pdf
3) Bed head signs (https://www.tracheostomy.org.uk/storage/files/NTSP_GREEN_Algorithm_Double_sided_2.pdf)